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NEUROLOGY FOLLOW-UP CONSULTATION NOTE

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DOCUMENT INFORMATION

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Document Title: Rohan Iyer Neurology Follow-up
Document Type: FOLLOWUP
Document Date: 12 March 2025
Department: Department of Neurology
Consulting Physician: Dr. Meenakshi Subramaniam, DM Neurology

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PATIENT INFORMATION

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Patient Name: Rohan Iyer
Patient Identifier: NEW-PATIENT-001 (External Synthetic ID)
Date of Birth: 17-Nov-1997 (Age: 27)
Gender: Male
Blood Group: A+
Contact Number: 9876543210
Email: rohan.iyer@fictional.demo
Address: Flat 12B, Orchid Residency, Indiranagar, Bengaluru, Karnataka

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REASON FOR FOLLOW-UP

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Scheduled neurology follow-up visit, approximately eight months after initial consultation. Patient was first seen on 18 July 2024 for evaluation of recurrent headaches with migrainous features. An MRI Brain was subsequently performed and returned to clinic for review and ongoing management.

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PREVIOUS INVESTIGATIONS

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MRI Brain (with and without contrast) – dated 25 July 2024 (Accession No.: MRI-2024-0725-1148, Sanjeevan Neurosciences Clinic Imaging Centre):

RESULT: No acute intracranial abnormality identified. No infarction, haemorrhage, or space-occupying lesion. Normal MRI of the brain.

Incidental finding: Mild right maxillary sinus mucosal thickening (clinically insignificant).

Routine Blood Work (performed July 2024):
CBC: Within normal limits.
ESR: 8 mm/hr (normal).
CRP: 0.4 mg/L (normal).
TSH: 2.1 mIU/L (normal, reference 0.4-4.0).

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INTERVAL HISTORY

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Since the initial consultation in July 2024, the patient reports a meaningful improvement in headache frequency and severity. Over the past eight months:

- Headache frequency has reduced from 2-3 episodes per week to approximately 2-3 episodes per month.
- Headache intensity has decreased from moderate-severe (6-7/10 on VAS) to mild-moderate (3-4/10 on VAS).
- Duration of individual episodes has also shortened (now typically 2-4 hours, previously 4-12 hours).
- Photophobia is now rare and limited to the more intense episodes only.

The patient attributes the improvement to:

- Implementation of sleep hygiene recommendations (now sleeping approximately 7 hours per night, up from 5-6 hours).
- Reduced screen time during non-work hours.
- Maintaining a regular exercise routine (walking 30 minutes, 4-5 days/week).
- Reduced caffeine intake (from 3 cups to 1 cup of coffee per day).
- Maintaining a headache diary as advised.

The patient was started on a low-dose preventive medication (Propranolol) by Dr. Subramaniam during a brief telephone review in November 2024, after headache frequency had plateaued at approximately once per week.

No episodes of sudden thunderclap headache. No seizures. No visual aura. No focal neurological symptoms. No syncope. No new symptoms reported.

CURRENT MEDICATIONS

Medication	Dose	Route	Frequency	Status	Notes
Propranolol	20 mg	Oral	OD (AM)	Active	Preventive; started Nov 2024
Paracetamol	500 mg	Oral	PRN	Active	Acute headache relief; max 2g/day

No other prescription medications.
No known drug allergies.

PHYSICAL EXAMINATION

General: Well-appearing, relaxed, in no acute distress.

Vitals:

Blood Pressure: 116/72 mmHg (right arm, sitting)
Heart Rate: 66 bpm, regular (note: on Propranolol)
Respiratory Rate: 14/min
Temperature: 36.7°C
Weight: 73 kg (previous: 74 kg)
BMI: 23.8 kg/m²

NEUROLOGICAL EXAMINATION

Mental Status: GCS 15/15. Alert, oriented to time, place, and person.
Cranial Nerves: CN I-XII intact. PERRL 3 mm bilaterally. No papilloedema.
Visual fields full. Facial symmetry preserved.
Motor: Power 5/5 all four extremities. Tone normal.
Sensory: Intact to light touch, pinprick, vibration, proprioception.
Reflexes: 2+ bilaterally and symmetric. Plantars flexor bilaterally.
Coordination: Finger-to-nose and heel-to-shin normal. No dysmetria.
Gait: Normal gait and tandem walk.

Neurological examination remains unremarkable with no focal deficit.

ASSESSMENT

27-year-old male with a diagnosis of migraine without aura, initially presenting in July 2024 with recurrent episodic headaches. MRI Brain (July 2024) was normal, excluding secondary intracranial pathology.

There has been a clinically significant improvement in headache frequency (from 2-3/week to 2-3/month) and severity over the past eight months. The improvement is attributable to a combination of lifestyle modifications and low-dose Propranolol prophylaxis.

Current headache burden is manageable and the patient is satisfied with his symptom control.

No red-flag neurological features. Neurological examination remains normal.

PLAN

1. MEDICATION:

- Continue Propranolol 20 mg OD (morning) for migraine prophylaxis.
- Continue Paracetamol 500 mg PRN for breakthrough headaches.
- Monitor resting heart rate; patient counselled to report bradycardia symptoms (dizziness, fatigue, presyncope).

2. LIFESTYLE:

- Continue current lifestyle modifications (sleep hygiene, exercise, reduced caffeine, limited screen time).
- Continue maintaining headache diary.

3. FOLLOW-UP:

- Review in 6 months (approximately September 2025).
- If headaches remain well-controlled at next visit, consider gradual tapering of Propranolol.
- If headache frequency increases or new symptoms develop, return sooner.
- No repeat neuroimaging indicated at this time given clinical improvement and previously normal MRI.

4. PATIENT COUNSELLING:

- Reassured that brain MRI was normal and the headache pattern is consistent with primary migraine.
- Advised to present to the emergency department if experiencing sudden severe headache ("worst headache of life"), seizures, visual loss, or any new focal neurological symptoms.

PHYSICIAN SIGNATURE

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Electronically signed by:

Dr. Meenakshi Subramaniam, DM Neurology
Senior Consultant, Department of Neurology
Sanjeevan Neurosciences Clinic, Bengaluru

Date: 12 March 2025

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